

Hapacol Paracetamol 500mg - PIL
Size: W5.5" x H8"

QUALITATIVE AND QUANTITATIVE COMPOSITION

Each tablet contains Paracetamol 500 mg.

PRODUCT DESCRIPTION: White to off-white, round effervescent tablets, plain on both sides, undamaged sides and edges. When dissolved or dispersed in water, the tablet effervesces to produce a slightly opaque solution with scent and sweet taste.

PHARMACODYNAMICS

Pharmacotherapeutic group: Other Analgesics and Antipyretics – Anilides.

ATC Code – N02BE01

Paracetamol is an effective analgesic and antipyretic agent. The drug has no effect on the cardiovascular and respiratory systems, and it does not cause gastric irritation or bleeding like salicylates.

PHARMACOKINETICS

Paracetamol is readily absorbed from the gastrointestinal tract with peak plasma concentrations occurring 30 minutes to 2 hours after ingestion. It is distributed in most body tissues; it crosses the placenta and is present in breast milk. Plasma protein binding is negligible at usual therapeutic concentrations but increases with increasing concentration. The elimination half life varies from about 1 to 3 hours.

Paracetamol is metabolised in the liver and excreted in the urine mainly as the glucuronide and sulphate conjugates. Less than 5% is excreted unchanged as paracetamol. A minor hydroxylated metabolite which is usually produced in very small amounts by mixed-function oxidases in the liver and which is usually detoxified by conjugation with liver glutathione, may accumulate following paracetamol overdosage and cause liver damage.

INDICATIONS

For the relief of mild to moderate pain including headache, migraine, muscle ache, dysmenorrhoea, sore throat, musculoskeletal pain, toothache and pain of osteoarthritis.

For the relief of fever.

RECOMMENDED DOSAGE

Adults (including the elderly) and children aged 12 years and over:

1 to 2 tablets (500mg to 1000mg paracetamol), dissolved in at least half a tumbler of water every 4 to 6 hours as required. Maximum 8 tablets (4000mg paracetamol) in any 24 hours period.

Maximum duration of continued use without medical advice: 3 days.

HAPACOL ***Effervescent*** ***Tablet 500mg***

Maximum daily dose: 60mg/kg to be administered in divided doses of 10-15mg/kg throughout the 24-hour period.

Children 9 to 11 years: 1 tablet (500mg) dissolved in at least half a tumbler of water. Maximum 4 tablets (2000mg paracetamol) in any 24 hours period.

Children under 9 years: Not recommended for children under the age of 9 years.

ROUTE OF ADMINISTRATION: For oral administration.

CONTRAINDICATIONS

Hypersensitivity to paracetamol or any of the other ingredients.

WARNINGS AND PRECAUTIONS

The tablets also contain aspartame (a source of phenylalanine) and so should not be taken by people with phenylketonuria.

Care is advised in the administration of paracetamol to patients with severe renal or severe hepatic impairment. Patients with alcohol dependence could also be at risk in taking paracetamol. The hazards of overdose are greater in those with non-cirrhotic alcoholic liver disease.

Each tablet contains sodium and may be harmful to people on a low sodium diet.

Do not exceed the recommended dose.

Immediate medical advice should be sought in the event of an overdose even if you feel well, because of the risk of delayed serious liver damage.

This preparation contains PARACETAMOL. Do not take any other paracetamol containing medicines at the same time.

Allergy alert:

Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash.

These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

Keep out of the reach of children.

Hapacol Paracetamol 500mg - PIL

Size: W5.5" x H8"

INTERACTIONS WITH OTHER MEDICAMENTS

Alcohol reduces liver capacity to deal with paracetamol. The speed of absorption of paracetamol may be increased by metoclopramide or domperidone and absorption reduced by cholestyramine.

The anticoagulant effect of warfarin and other coumarins may be enhanced by prolonged regular use of paracetamol with increased risk of bleeding; occasional doses have no significant effect.

STATEMENT ON USAGE DURING PREGNANCY AND LACTATION

Epidemiological studies in human pregnancy have shown no ill effects due to paracetamol used in the recommended dosage, but patients should follow the advice of their doctor regarding its use.

Paracetamol is excreted in breast milk but not in a clinically significant amount. Available published data do not contraindicate breast feeding.

EFFECTS ON ABILITY TO DRIVE AND USE MACHINES

None known.

UNDESIRABLE EFFECTS

Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.

Side effects are usually mild, although haematological reactions have been reported. Rashes and other allergic reactions occur occasionally. There have been isolated reports of thrombocytopenia, purpura, methaemoglobinaemia and agranulocytosis.

OVERDOSE AND TREATMENT

Liver damage is possible in adults who have taken 10g or more of paracetamol.

Ingestion of 5g or more of paracetamol may lead to liver damage if the patient has risk factors (see below).

Risk Factors:

If the patient

a). Is on long term treatment with carbamazepine, phenobarbitone, phenytoin, primidone, rifampicin, St John's Wort or other drugs that induce liver enzymes.

Or

b). Regularly consumes ethanol in excess of recommended amounts.

Or

c). Is likely to be glutathione deplete e.g. eating disorders, cystic fibrosis, HIV infection, starvation, cachexia.

Symptoms

Symptoms of paracetamol overdosage in the first 24 hours are pallor, nausea, vomiting, anorexia and abdominal pain. Liver damage may become apparent 12 to 48 hours after ingestion. Abnormalities of glucose metabolism and metabolic acidosis may occur. In severe poisoning, hepatic failure may progress to encephalopathy, haemorrhage, hypoglycaemia, cerebral oedema, and death. Acute renal failure with acute tubular necrosis, strongly suggested by loin pain, haematuria and proteinuria, may develop even in the absence of severe liver damage. Cardiac arrhythmias and pancreatitis have been reported.

Management

Immediate treatment is essential in the management of paracetamol overdose. Despite a lack of significant early symptoms, patients should be referred to hospital urgently for immediate medical attention. Symptoms may be limited to nausea or vomiting and may not reflect the severity of overdose or the risk of organ damage. Management should be in accordance with established treatment guidelines, see BNF overdose section.

Treatment with activated charcoal should be considered if the overdose has been taken within 1 hour. Plasma paracetamol concentration should be measured at 4 hours or later after ingestion (earlier concentrations are unreliable). Treatment with N-acetylcysteine may be used up to 24 hours after ingestion of paracetamol, however, the maximum protective effect is obtained up to 8 hours post-ingestion. The effectiveness of the antidote declines sharply after this time. If required the patient should be given intravenous N-acetylcysteine, in line with the established dosage schedule. If vomiting is not a problem, oral methionine may be a suitable alternative for remote areas, outside hospital. Management of patients who present with serious hepatic dysfunction beyond 24h from ingestion should be discussed with the NPIS or a liver unit.

STORAGE CONDITIONS: Store in dry places, not exceeding 30°C, protect from light.

DOSAGE FORM AND PACKAGING AVAILABLE:

Box of 4 strips x 4 effervescent tablets.

DATE OF REVISION OF PI: December 30, 2024

Manufactured by:

DHG PHARMACEUTICAL JOINT-STOCK COMPANY - DHG Pharmaceutical plant branch in Hau Giang
Lot B2 - B3, Tan Phu Thanh industrial zone - phase 1, Tan Phu Thanh commune, Chau Thanh A district, Hau Giang province, Vietnam
Tel: (+84,293) 3953454 • Fax: (+84,293) 3953555